Project Partner Section 56(1) Drug-Checking Exemption Application Guide

A practical handbook for SUAP partner sites preparing CDSA s.56(1) drug-checking exemption applications

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A Note Before You Begin

This guide walks you through every part of a Health Canada Section 56(1) drug-checking exemption application. It covers five things:

- the Health Canada application form (field by field)
- the supporting documents you send with the form (document by document)
- the follow-up questions Health Canada usually asks during review
- what you have to do after you get approved
- Health Canada's own list of what an application must contain, sent to the project by the Exemptions Division on October 15, 2026 (Appendix I)

The form examples come from a real partner application that was approved. All site names and personal details have been removed.

This is not legal advice. Health Canada decides each application case by case. You will likely go back and forth with them more than once. Talk to your own lawyer if you have legal questions, and write to the Office of Controlled Substances any time you are unsure.

This guide is current as of November 2026. Three things changed between the May 2026 edition and this one, all on Health Canada's written word: there is no education requirement for the Principal Investigator, the RPIC or the A/RPICs (Part II.3); the RPIC's criminal record check must be less than twelve months old at submission, not six (Part III.7); and the Institution/Company box in Section 2.3 takes the name of the drug checking site or service, not the legal name of the organization (Part II.5). Health Canada's full requirements list is reproduced in Appendix I.

Part I — Foundation

I.1 Who This Guide Is For

This guide is for directors, executive leaders, and compliance staff at SUAP partner sites. Use it when you are getting ready to:

- send a new Section 56(1) drug-checking exemption application to Health Canada,
- renew an existing one, or
- change one you already have.

This guide does not cover other rules. It does not cover Supervised Consumption Sites (which use CDSA Section 56.1), Urgent Public Health Need Sites (which use a class exemption), or provincial rules. If your site falls under one of those, your process will be different.

Your partnership with Western University is separate from your federal exemption. Western tests samples your site sends in under Western's own Section 56(1) exemption. Your site's own exemption covers what you do at your own location and during transport to an approved recipient — usually Western, a Health

Canada lab, or another exemption holder.

I.2 Fixed Site, Mobile Site, or Both

Health Canada exemptions cover one site at a time. If you have both a fixed building and a mobile outreach unit, you cannot put them on one application. You need two separate applications. Each gets its own decision letter and its own rules to follow afterwards.

If your organization runs (or plans to run) both, you will go through Parts II and III of this guide twice — once for the fixed site and once for the mobile unit. The form and the documents you send are mostly the same for both. The differences are:

- A few answers on the form change (mainly the use address, mailing address, and storage address).
- Mobile sites need extra documents: a vehicle photo, vehicle floor plan, route maps, stop addresses, outreach materials, and a list of who staffs the unit.

Part III flags every mobile-only item where it applies.

I.3 Legal Background in Plain Words

Some things you do during drug checking would normally break the law. Without a Health Canada exemption, possessing, transferring, or testing illegal substances breaks several parts of the *Controlled Drugs and Substances Act* (CDSA). The exact sections are 4(1), 5(1), 5(2), and 7(1) of the CDSA, and 6(1) and 6(2) of the Precursor Control Regulations.

Section 56 of the CDSA lets the Minister of Health give exemptions from those rules.

- **Subsection 56(1)** is the general exemption power. It is what stand-alone drug-checking sites apply for. This guide is about Section 56(1) applications.
- **Section 56.1** is a separate provision for Supervised Consumption Sites only. If your site is a licensed SCS, drug checking is already covered by your existing 56.1 exemption. You do not need a 56(1).

A Section 56(1) exemption names the people who are allowed to do drug-checking activities. That usually includes the Responsible Person in Charge (RPIC), Alternate RPICs (A/RPICs), trained staff, and clients. These people are only protected when the activity is for drug checking or disposal, and when it happens at the address listed in the exemption.

I.4 How to Send Your Application and What to Expect

Where to send a new application. Email your full application package to exemption@hc-sc.gc.ca. This is the address Health Canada lists on its current Section 56(1) scientific-purposes page (checked November 2026), and the address its Exemptions Division gives in its own requirements list (Appendix I). The live URL is in Appendix G.

Where to send everything else. Renewals, monthly reports, loss-or-theft notices, and any other emails about your file all go to the same address: exemption@hc-sc.gc.ca.

Mailing address (only if you need to send a paper copy with a wet signature). Exemptions Division, Office of Controlled Substances, Health Canada, 161 Goldenrod Drwy, AL0300B, Ottawa ON K1A 0K9. (Format taken from Health Canada's current published page, November 2026.)

Phone. Health Canada's exemptions page lists no phone number for the Exemptions Division, but its correspondence with partner sites has given 343-990-4862 (March 2026). Email is the reliable route: send questions to exemption@hc-sc.gc.ca, and ask for a call in the email if you need one.

Privacy questions. Send privacy questions about your file to exemption@hc-sc.gc.ca. If you want to make a formal Access to Information and Privacy (ATIP) request, file it through the Government of Canada's online ATIP portal.

How long it takes. Health Canada says the average processing time is **70 calendar days** once they have all the information they need. That number does not include the back-and-forth where they ask for more details, which happens with almost every drug-checking application. In practice, partner sites have waited several months from submission to decision letter, and longer where the questions ran to several rounds.

How long the exemption lasts. Health Canada's website says exemptions are valid for **two years** from the date they are issued. In practice, drug-checking exemptions for partner sites have often been issued for shorter terms (one year is common), with renewals available. Plan your renewal around the expiry date written in your own decision letter, not the website's default.

The Controlled Substances Regulations. The new Controlled Substances Regulations (SOR/2025-242) were published in the Canada Gazette Part 2 on December 17, 2025 and came into force on October 1, 2026. They did not change the route for drug checking: Health Canada's Exemptions Division has continued to describe and issue subsection 56(1) drug checking exemptions since that date (its requirements list of October 15, 2026, and partner decision letters issued in October 2026), and this guide follows that process. Live URLs are in Appendix G.

Part II — The Health Canada Application Form, Field by Field

You will fill out one Health Canada form: *Application form for an exemption to use a controlled substance for scientific purposes*. The Office of Controlled Substances put it out in April 2018. It is 10 pages long and has 8 numbered sections, plus Appendix A (Import Order Authorization) and a checklist at the end. The current PDF is here: https://www.canada.ca/content/dam/hc-sc/documents/services/health-concerns/controlled-substances-precursor-chemicals/exemptions/ScientificExemptionApplicationFormEN-v1.pdf.

The project also has a copy of the blank form on file. Before you start this Part, open the form in Adobe Acrobat (or any PDF reader that lets you fill in fields). The walkthrough below follows the form page by page, so you can fill in each field as you read.

REDACTED EXAMPLE

The redacted answers shown in the FIELDTABLE blocks throughout Parts II and III are drawn from the approved Section 56(1) drug-checking exemption application of a SUAP partner site held on file with the project. All site-identifying details — organization name, addresses, names, telephone numbers, email addresses, dates, license numbers, and police service names — have been replaced with bracketed placeholders. The structural patterns, language conventions, and answer logic shown are accurate to the approved submission.

II.1 Page 1 — Privacy Notice (no fields to fill)

Page 1 is a Privacy Act notice. You do not need to fill in anything on this page. The notice tells you:

- The personal information on the form is protected by the federal *Privacy Act*.
- Health Canada is allowed to collect it under the CDSA.
- They will use it to review your application. They may also use it for reporting, research, planning, audits, and evaluation.
- In some narrow cases (for example, when the law requires it), they can share it with police or other parties without your consent.
- If you do not provide the information they ask for, they can refuse to process the application.

For questions about how Health Canada handles your information, write to exemption@hc-sc.gc.ca (per the current published exemptions page). Read the page once and move on.

II.2 Page 2 — Section 1: Application Type

Section 1 asks what kind of application you are sending. Pick one of the six choices. If it is not a brand-new application, write down the authorization number from your existing exemption.

HC Form Field	What HC Asks	Redacted Approved Example
■ New	First-time application for this site, this PI, this substance scope.	■ — selected for a new drug-checking application at a previously unexempted site.
■ Amendment of a valid exemption	Material change to an exemption already in force (e.g., adding a substance class, changing the RPIC, modifying the storage configuration) without changing the exemption term.	Not selected (typical for new sites).
■ Extension (no additional quantities)	Continuing an existing exemption beyond its expiry date with no change in authorized quantity.	Not selected.
■ Extension (additional quantities)	Continuing an existing exemption AND increasing the authorized quantity.	Not selected — drug-checking does not authorize fixed quantities, samples come from clients.

HC Form Field	What HC Asks	Redacted Approved Example
■ Cancellation of a valid exemption	Surrendering an exemption before its expiry date.	Not selected.
■ Transfer of controlled substances from one researcher to another within an institution	Reassigning responsibility for an exemption (with substances on hand) from one PI to another.	Not selected.
Current authorization number (if not new)	The HC authorization number of the existing exemption being amended/extended/cancelled/transferred. Format YY-NNNNNN-NNN.	Left blank for a new application.

■ HC FLAG

Drug-checking applications almost never use "Extension (additional quantities)." That choice is for research labs that hold a fixed amount of substance. Drug-checking exemptions cover substances that clients bring in — there is no inventory to increase. For a renewal where nothing has changed, use "Extension (no additional quantities)." If your site, RPIC, or scope is changing at renewal, use "New." If you are not sure, email the Exemptions Division before you submit.

II.3 Page 2 — Section 2.1: Principal Investigator

Section 2.1 names the Principal Investigator (PI) on the application. This is the individual leading the application on behalf of the applicant organization. The PI named on the form does not have to be the same individual as the Responsible Person In Charge (RPIC) named in your supporting documents — these can be different people with different roles (for example, the PI may be an institutional research lead and the RPIC may be the on-site clinical lead). Whoever you list, make sure their contact information matches what appears in the rest of the package.

The form prints a note directly under Section 2.2 saying the minimum requirement of study for the applicant is a B.Sc. in an appropriate field. That note belongs to the scientific-purposes form the drug checking process borrows, and it does not apply here. Health Canada's Exemptions Division confirmed in writing on September 17, 2026 that there are no education requirements for the applicant (the Principal Investigator on the form), the RPIC or the A/RPICs for drug checking services, and that the form contains fields that may not be relevant to a drug checking service. The same letter confirmed that co-applicant organizations are allowed. The RPIC's requirements are operational: a ten-year employment history with locations and a Canadian criminal record check (see III.7). Health Canada's own wording for the person who signs is "the individual legally responsible for the site" (Appendix I, section H).

HC Form Field	What HC Asks	Redacted Approved Example
Mr. / Mrs. / Dr.	Salutation — choose one.	■ Dr.
Surname (last name)	Family name of the PI.	[PI Surname]

HC Form Field	What HC Asks	Redacted Approved Example
Given Name(s)	First and middle names of the PI.	[PI Given Name]

II.4 Page 2 — Section 2.2: Title and Qualifications

Section 2.2 records the PI's credentials, contact details and an alternate contact. There is no education requirement for drug checking (see II.3), so tick the credential boxes that apply and leave the rest. If the PI is a practitioner (M.D., D.V.M., D.M.D., D.D.S.), also write in their provincial Practitioner Permit Number. Fill in the PI's field of study, phone number and email. The alternate contact is the person Health Canada writes to day to day: the one who answers questions, files the monthly reports and handles renewals.

HC Form Field	What HC Asks	Redacted Approved Example
■ B.Sc. ■ M.Sc. ■ Ph.D. ■ M.D. ■ D.V.M. ■ D.M.D. ■ D.D.S.	Credentials held by the PI, if any. Select all that apply. There is no education requirement for drug checking (II.3).	■ M.D. (other credentials at the PI's discretion)
Practitioner Permit Number	Provincial licence number issued by the PI's regulatory college (required for M.D., D.V.M., D.M.D., D.D.S.).	[Provincial Practitioner Permit Number]
Field of Study	The PI's primary clinical or scientific field.	[Field of Study, e.g., Internal Medicine]
Telephone Number	Direct line for the PI.	[PI Direct Telephone]
Email Address	Direct email for the PI.	[PI Direct Email]
Alternate Contact Name	A second person at the applicant organization who can be reached by HC if the PI is unavailable. Typically the day-to-day program lead.	[Alternate Contact Name]
Alternate Contact Email	Email for the alternate contact.	[Alternate Contact Email]

■ HC FLAG

The Practitioner Permit Number is the number from the PI's provincial regulatory college (for example, CPSO in Ontario or CPSBC in British Columbia). It is not a federal licence, not a CPSI number, and not a CMA membership number. If the PI is licensed in a province other than where your site operates, write the licensing province next to the permit number. This stops Health Canada from following up to ask which province the licence is from.

II.5 Page 2 — Section 2.3: Address (where the substance will be used)

Section 2.3 is the **Site address** — the physical place where you will be working with illegal substances. For a fixed site, this is the building (and the specific suite or unit, if any) where you receive samples, test them, store them while waiting for pickup, and destroy them. For a mobile site, this is where the vehicle is parked when not in use — usually your organization's main address. You describe the route and the stops in your supporting documents (see Part III). Health Canada's instruction for this section (Appendix I, section H) is to put the name of the drug checking site or service in the Institution/Company box, and the address of the site where the service would be offered.

HC Form Field	What HC Asks	Redacted Approved Example
Institution / Company	The name of the drug checking site or service (Health Canada's instruction, October 2026). Use the same name on the protocols title page and in Section 3.	[Name of the drug checking site or service]
Department / Faculty	Subdivision of the organization, if applicable. For a community health centre the program name is appropriate.	[Drug Checking Program]
Street Address	Street number and street name of the Site.	[Site Street Address]
Room Number	Specific suite, unit, or room number within the building. Required if the building houses unrelated tenants.	[Site Suite/Unit Number]
City	City.	[City]
Province	Province (use the two-letter postal abbreviation).	[Province]
Postal Code	Canadian postal code in standard "A1A 1A1" format.	[Postal Code]

Fixed Site	Mobile Site
The building address where drug checking happens. Include the suite or unit number if your building has more than one tenant.	The address where the vehicle is parked when not in service (usually your organization's main address). You describe the route and the stops in your supporting documents (see Part III).

II.6 Page 3 — Section 2.4: Mailing Address (where the substance will be shipped)

Section 2.4 is the mailing address. This is where Health Canada sends any paper mail about your exemption — like the decision letter or any printed notices. The form's heading says "where the substance will be shipped," but in drug checking, no one ships you substances. Clients bring them in. So Health Canada reads this field as your postal mailing address. Most applicants tick "Same as above address" and skip the rest of the field.

HC Form Field	What HC Asks	Redacted Approved Example
■ Same as above address	Check if the mailing address is identical to the Section 2.3 Site address.	■ Checked.
Institution / Company	Used only if mailing address differs from Site address.	(left blank when "Same as above" is checked)
Department / Faculty	As above.	(blank)
Street Address, Room Number, City, Province, Postal Code	As above.	(blank)

II.7 Page 3 — Section 2.5: Storage Address (where the substance will be stored)

Section 2.5 is where samples are stored. For most fixed sites, the storage address is the same as the use address — samples are kept in a locked compartment in the same room where you test them. For mobile sites, the storage address must be your fixed home base. **Substances cannot stay in the vehicle when the vehicle is not in service.**

HC Form Field	What HC Asks	Redacted Approved Example
■ Same as above address provided in section 2.3	Check if storage occurs at the Site address.	■ Checked (for fixed sites where storage is on-Site).
■ Same as above address provided in section 2.4	Check if storage occurs at the mailing address.	(only used if 2.4 differs from 2.3)
Institution / Company, Department/Faculty, Street, Room, City, Province, Postal Code	Used only if storage address differs from both 2.3 and 2.4.	(blank for fixed sites; for mobile sites, the parent organization's address where the vehicle is parked overnight)

■ **HC FLAG**

For mobile applications, **do not tick** "Same as above" if Section 2.3 already shows the vehicle's home base. Instead, name the exact room or locked office where samples are kept when the vehicle is not in service. In your supporting protocols (see Part III), state plainly that no substances stay in the vehicle outside operating hours. If you skip this, Health Canada will almost always come back and ask about overnight storage.

II.8 Page 3 — Section 3: Project or Study Description

Section 3 names your project, lists the documents you are sending with the form, and gives space to describe how you will use the substances. The Project Title on the form **must match exactly** the title at the top of your operational protocols document (see Part III, Section III.1). If the two titles do not match, Health Canada will almost always come back and ask about it.

HC Form Field	What HC Asks	Redacted Approved Example
Project Title (same as protocol)	Project name. Must match the title on the attached protocol document exactly.	[Applicant Organization]'s Drug Checking Program — Scatr Device
■ Protocol attached	Check if the operational protocol is included as an attachment to this application.	■ Checked.
■ Protocol previously submitted, if not amended	Check only at renewal time if the protocol is unchanged from a prior submission.	Not checked for new applications.
■ Approval of the Animal Care Committee (for in vivo studies)	Required only if substances will be administered to animals.	Not checked — no animal use in drug checking.
Please provide any further information	Free text. Use this for context HC needs to understand the application: regulatory background, why a new exemption is being sought, who the population served is, and how the project fits into the broader site operation. 200–500 words is typical.	[Free text paragraph: site's pre-existing drug-checking context (e.g., prior CTS or SCS authorization), reason a new s.56(1) is being sought, days/hours of operation, geographic catchment, and the program's relationship to Western University and Scatr.]

REDACTED EXAMPLE

The redacted approved "further information" paragraph from the partner application on file reads (with site-specific terms in brackets):

"[Applicant Organization]'s drug-checking program currently runs under the exemption of the Consumption and Treatment Services, that is slated to close at the end of [Month, Year]. After consultation with the community being served at the CTS, it was determined that the drug-checking program would be well-received if provided [N] days per week in a [fixed/mobile] format. This requires a new exemption. [Applicant Organization] will also be applying for a [fixed/second] exemption to provide drug checking out of our [clinical hub / mobile unit] [N–N] days per week. Our community has noted that testing offered only at one location is a significant barrier to those seeking this service, that has become a heavily relied-on service to test their substances and has proven to save lives."

II.9 Page 4 — Brief Description of the Use of the Substance

Page 4 has a short free-text box for the *use* of the substance. This is not the same as the project description in Section 3. Here, you answer one question: "What will the controlled substances be used for at this site?" The answer is short and looks roughly the same across all drug-checking applications.

HC Form Field	What HC Asks	Redacted Approved Example
Please provide a brief description of the use of the substance	One- or two-sentence statement of what activities the substances will be used for.	"Clients bring in their own street-acquired / unregulated substances to test, utilizing the Scatr device."
Please provide the reason for requiring an extension, cancellation or transfer of responsibility (if applicable)	Used only for non-new applications. Explain the change being sought.	Left blank for new applications.

II.10 Page 4 — Section 4: In Vivo Administration (NOT APPLICABLE for drug checking)

Section 4 is about giving controlled substances to animals. **None of Section 4 applies to drug checking** because there are no animals involved. The form itself gives a hint at the top: "In vitro utilization / In vivo administration. (*Please skip to section 5)." In other words, an in-vitro-only application skips the animal questions.

For drug checking, leave the whole In Vivo block blank. If the in vitro / in vivo radio buttons need an answer in your PDF reader, pick "In vitro utilization."

The checklist on page 10 (Section II.17 below) also confirms Section 4 stays unchecked when there are no animals.

II.11 Page 5 — Section 4.1: Controlled Substance(s) (NOT APPLICABLE for drug checking)

Section 4.1 asks for dosing details for animal studies (starting dose, maintenance dose, how often, total dose per animal). **None of this applies to drug checking** — you are not dosing anything, and there are no animals. Leave it blank.

II.12 Pages 5–6 — Section 5: Supplier Of The Controlled Substance (NOT APPLICABLE for drug checking)

Section 5 asks about who you buy controlled substances from. This is meant for research labs that order pure substances from a chemical supplier. **It does not apply to drug checking.** You do not buy substances. Substances come from your clients, in unknown form, and are returned to the client (or destroyed) after the test.

Leave the whole supplier block blank — brand name, concentration, quantity, supplier address, and contact details. Leave the Foreign Supplier checkbox unchecked. You are not importing anything.

The checklist on page 10 (Section II.17 below) also confirms Section 5 stays unchecked for drug-checking applications.

II.13 Page 6 — Section 6: Physical Security

Section 6 is one free-text box. The form heading reads: "Please provide a description of the physical storage and security measures to be used." Below it, Health Canada writes: "Security must meet the requirements of the 'Directive on Physical Security Requirements for Controlled Substances', available on the Health Canada website."

This is the most important free-text field on the whole form. Health Canada looks at your security set-up to decide whether they can approve your application at all. The form box is too small to hold a full security plan. Write a short summary in the box and point Health Canada to the full Security Plan in your supporting documents.

HC Form Field	What HC Asks	Redacted Approved Example
Please provide a description of the physical storage and security measures to be used	Summary of physical security on Site, with a pointer to the detailed policy in the supporting documentation package.	"Please refer to [Applicant Organization]'s Drug Checking Program protocols — Section 1.2(b) Security Plan — for a description." (Submit the full Security Plan as part of the operational protocols document; see Part III, Section III.1.)

■ HC FLAG

The "Directive on Physical Security Requirements for Controlled Substances" is referenced in Health Canada's instruction beneath Section 6. Health Canada publishes it on its Physical Security Directive page (URL in Appendix G). It was written for licensed dealers and sets eleven security levels according to the type and quantity of controlled substances stored. A drug checking site holds no inventory, only client samples for a short time, and the approved partner applications on file met Health Canada's security requirements with the security plan in the protocols document (1.2(b)) and a labelled floor plan, which is what the Exemptions Division's own requirements list asks for (Appendix I, section C). If you want to cite the directive in your plan, read the page first and keep to what applies. Your Plan must name the storage container (locked safe / secure cabinet / locked drawer), identify which staff hold keys or access codes, describe the access log, and describe what happens if a key or access credential is lost or stolen. Health Canada's requirements list (Appendix I, section C) sets out the four things the security material must contain: the security measures and who has access to the room; who issues keys, fobs or cards, that records of distribution are kept, and when they are changed; how illegal substances are stored, including residues, substances awaiting disposal and substances left behind; and a floor plan with every security measure drawn on it.

II.14 Page 7 — Section 7: Declaration

Section 7 is where the Principal Investigator signs to take legal responsibility. It has three different declarations, one for each kind of application. You only sign one.

7.1 — New, Extension, or Amendment Declaration

Use this declaration for new applications, extensions, and amendments. The verbatim text on the form reads: *"I hereby declare that I am the principal investigator and that the controlled substance(s) will be used for scientific purposes mentioned in this application. I have read and understand the Directives on Physical Security Requirements of controlled substances and other requirements specified in the Controlled Drugs and Substances Act and its Regulations. The specified requirements are met, or will be met before I commence any activity associated with any exemption issued to me. I also agree to comply with any terms and conditions that may be specified in any exemption issued to me."*

HC Form Field	What HC Asks	Redacted Approved Example
Signature of Principal Investigator	Wet or digital signature of the named PI from Section 2.1.	[Wet/digital signature of PI]
Date (YYYY-MM-DD)	Date of signature, in ISO format.	[YYYY-MM-DD]

7.2 — Cancellation Declaration

Use this only when you are giving up an exemption. Three checkboxes ask what you are doing with any substances you have on hand. Drug-checking sites rarely use this part. If you do, "no remaining inventory" is the usual answer, because drug-checking sites do not keep a stockpile of substances.

7.3 — Transfer Declaration

Use this only when one PI is handing the exemption to another PI at the same organization. Both PIs sign and date. List any substances being transferred (and the amounts) in the table provided. Drug checking almost never uses this.

II.15 Page 8 — Section 8: Submission

Section 8 is informational — it tells the applicant where to submit the completed package. The verbatim text on the form reads: *"Please email this completed Application Form and required documents to the Office of Controlled Substances at hc.exemption.sc@canada.ca or mail to the following address: Exemptions Section, Authorizations Division, Office of Controlled Substances, Health Canada, AL 0300B, 161 Goldenrod Drwy, Ottawa ON K1A 0K9."*

Health Canada's current public-facing Section 56(1) scientific-purposes exemption page (verified live, November 2026) lists exemption@hc-sc.gc.ca as the single submission address for both new applications and all subsequent correspondence (renewals, monthly reports, loss/theft notifications, and RFI responses). The address embedded in the body of the form template itself (hc.exemption.sc@canada.ca) appears to be an older designation that has not been removed from the PDF form. Use the address listed on Health Canada's current page: exemption@hc-sc.gc.ca.

II.16 Page 9 — Appendix A: Import Order Authorization (NOT APPLICABLE for drug checking)

Appendix A is the *Import Order Authorization and Permit Application Form* (IOPAF). It is only used when you need to import a controlled substance from outside Canada because no Canadian supplier carries it. **It does not apply to drug-checking applications** — you do not import substances. Leave all of Appendix A blank and unsigned.

The checklist on page 10 (Section II.17 below) also confirms this row stays unchecked when you are not importing anything.

II.17 Page 10 — Application Checklist

The last page of the form is a checklist. Health Canada uses it to make sure your package is complete before they open the file for review. If something is missing, they put your file on hold until you send the missing item.

The checklist has two parts: "Completed Information" (what is filled in on the form itself) and "Attachments" (the documents you send with the form).

For a drug-checking application, the table below shows which boxes are usually checked and which are usually left unchecked. This pattern is consistent across the approved applications on file.

Checklist Item	What HC Wants Confirmed	Drug-Checking Standard Answer
Section 1 — Application type	Confirms Section 1 is filled in.	■ Checked.

Checklist Item	What HC Wants Confirmed	Drug-Checking Standard Answer
Section 2 — Address etc.	Confirms applicant identification, addresses (use, mailing, storage) are filled in.	■ Checked.
Section 3 — Project title and description	Confirms project title (matching the protocol), protocol attachment, and use description are filled in.	■ Checked.
Section 4 — Animals/dosing	Confirms animal in vivo information is filled in.	■ Unchecked — no animal use.
Section 5 — Supplier purchase details	Confirms substance procurement details are filled in.	■ Unchecked — no purchased substances.
Section 6 — Storage and security	Confirms physical security description (and reference to the detailed Security Plan) is filled in.	■ Checked.
Section 7 — Declaration signed and dated	Confirms PI declaration is wet/digitally signed and dated.	■ Checked.
Attachment: Copy of the protocol	Confirms operational protocol is attached or previously submitted unamended.	■ Checked (operational protocols document — see Part III, Section III.1).
Attachment: Appendix A (Foreign Supplier IOPAF)	Confirms IOPAF is attached when required.	■ Unchecked — no foreign supplier.
Attachment: Animal Care Committee approval	Confirms ACC approval is attached when required.	■ Unchecked — no animal use.

■ HC FLAG

A common Health Canada follow-up is when the page-10 checklist is left completely blank. This puts your file on hold until you confirm which sections you meant to leave out. Always fill in the checklist. Even rows you intentionally skip should have the box marked one way or the other.

Part III — Supporting Documentation Package, Document by Document

The supporting documentation package is everything you send with the Health Canada application form. The form on its own is not enough — Health Canada will not open your file for review until the full package arrives.

The layout of the package is well-established across Western's partner network and is based on a redacted approved application held on file.

- Sections III.1 through III.7 cover the documents needed for **both fixed and mobile** applications.
- Section III.8 covers the **extra documents only mobile applications need**.

Fixed-site applicants need III.1 through III.7. Mobile-site applicants need III.1 through III.8.

III.1 The Operational Protocols Document

This is the most important document in your package. It is a policies-and-procedures document that describes, step by step, every activity at your site that involves illegal substances. The Health Canada form refers to this document by name in Section 3 (project description) and Section 6 (physical security).

A redacted approved partner protocols document held on file is organized as below. **Use this exact section numbering and structure in your own protocols document.** Health Canada reviewers expect this layout. When your form points to (for example) "protocols Section 1.2(a)," the reviewer can find it right away.

- Title page: organization name, document title (Drug Checking Program — Policies and Procedures), application reference (Class 56(1)), date.
- 1.1 Background and the Scatr device — what drug checking is, what the device does, age restriction (18+ per Western HSREB), data collection requirement.
- 1.1(a) Drug checking data reports — the six required monthly report elements; submission to exemption@hc-sc.gc.ca by the 15th of each month.
- 1.1(b) Staffing — minimum staffing requirements; FTE counts; conditions under which the program operates.
- 1.1(c) Hours of operation — concrete weekly schedule.
- 1.2 Staff roles and responsibilities — section heading.
- 1.2(a) Drug Checking Program Lead — responsibilities of the program lead.
- 1.2(b) Security Plan — physical access controls, key/access-card distribution, alarm response, lost-credential response.
- 1.2(c) Drug Checking Program Peer — responsibilities of peer worker(s).
- 1.2(d) Staff training requirements — required training topics, sign-off process.
- 1.2(e) Confidentiality — client confidentiality and data handling.
- 1.3 Equipment — the Scatr Series One device and any supplementary equipment (e.g., immunoassay strips).
- 1.4 Disclaimer script for clients — verbatim language given to every client before testing.
- 1.5 Substance preparation for analysis — how a sample is prepared for a Raman or NIR scan.
- 1.6 Infection control routine practices — handling of substances that may be biologically contaminated; PPE; surface decontamination.

- 1.7 Storage, substances left behind, handover to Western — section heading.
- 1.7(a) Managing substances voluntarily submitted for offsite quantitative/trace testing at Western.
- 1.7(b) Storage — where samples are stored on Site, who has access, and for how long.
- 1.7(c) Handover — chain-of-custody for samples being transported to Western (lockbox, manifest, courier).
- 1.7(d) Protocol for unidentified substances left behind — the immediate response.
- 1.7(e) Prevention of unidentified substances left behind — operational design choices that minimize occurrence.
- 1.7(f) Management of unidentified substances left behind — log, police notification within 24 hours, pickup.
- 1.8 Loss or theft of substances — three notifications within 24 hours (local police, OCS at exemption@hc-sc.gc.ca, HC Loss or Theft Report Form); per-substance log.

III.2 Letter of Information and Consent

The Letter of Information and Consent (LOI) is the document every drug-checking client gets at intake. It explains the program in plain language: what data is collected, how the substance will be handled, what the risks and benefits are, and how to contact the research team or the Research Ethics Board with concerns.

The LOI is required for two reasons:

1. Health Canada expects to see proof that clients are informed participants, not test subjects without their knowledge. 2. Western University's Health Sciences Research Ethics Board (HSREB) has approved one LOI template that covers the whole SUAP project. Partner sites must use that template (with their own contact details filled in) to be covered by Western's umbrella study protocol.

The current Western HSREB-approved LOI is version 3, dated October 7, 2026 and approved October 13, 2026. It is four pages long.

- **Page 1** introduces the study, names the partners (Western, Scatr Inc., your site), and explains the anonymous client code system.
- **Page 2** covers risks and benefits, voluntary participation, the right to withdraw, and confidentiality.
- **Page 3** discloses the conflict of interest with Scatr Inc., names the Principal Investigator and study contacts, and gives the Western HSREB office contact for ethics concerns.
- Page 1 — Title: "Letter of Information and Consent"; brief description of the drug-checking research study; named participating organizations; explanation of the anonymous client code system used to track results across visits.
- Page 1 — Statement that participation in drug-checking does not require participation in the research data collection (the two are operationally bundled but conceptually separable).
- Page 2 — Risks and benefits of participation, framed honestly (no overstated benefit claims).

■ Page 2 — Voluntary participation language and explicit right to withdraw at any time without consequence.

■ Page 2 — Confidentiality and data handling: anonymous client code, no personal identifiers stored, data shared only with Western and Scatr for research purposes.

■ Page 3 — Conflict-of-interest statement regarding the Scatr Inc. relationship.

■ Page 3 — Named contacts: PI (Prof. François Lagugné-Labarthet, flagugne@uwo.ca, 519-661-2111 x81006), Project Manager (Cameron Brown, cbrown58@uwo.ca, 226-238-9970), and Western HSREB Research Ethics Office.

REDACTED EXAMPLE

The full current Western HSREB-approved LOI template (version 3, Version Date: Oct 7, 2026) is reproduced word-for-word in Appendix H of this guide. ***Do not write your own LOI from scratch.*** *You also do not need Health Canada to approve the LOI text — they accept it because Western's HSREB has already approved it under the umbrella study protocol you cite in your application.*

■ **HC FLAG**

Do not change the wording of the LOI. The document is approved under Western's HSREB study protocol. Any change beyond filling in your site's contact details needs a protocol amendment, which the Project Manager handles. If you want to change anything else (a greeting, where the logo sits), talk to the Project Manager before you print.

III.3 Site Floor Plan(s)

The site floor plans show Health Canada that your physical site has the security features described in your protocols document. The plans show:

- the room layout
- where the doors are and where the locks are
- where the safes are kept (one for samples going to Western, one for unidentified substances left behind)
- where any panic buttons or alarms are
- the path a client takes from arrival to drug-checking to leaving the site

For a fixed site, you usually need two floor plans:

1. A **building-level overview** showing where the drug-checking suite is in the larger building and how clients get to it from the main entrance. 2. A **detail plan of the drug-checking room itself**, with security features labelled.

■ Building-level floor plan: clearly identifies the drug-checking suite within the building; shows the path from the main entrance to the suite (locked doors, sign-in points); shows other tenants or program areas the drug-checking suite is adjacent to (so HC can assess proximity to non-program

activity).

- Drug-checking room detail plan: shows the drug-checking workstation, client-side and staff-side seating positions, location of the secure cupboard housing the samples-for-Western safe and the unidentified-substances safe, location of panic buttons (positions kept out of client sight), location of the sink and biohazard storage, and any other security features (cameras, motion sensors).
- Both plans labelled with: building address, suite/unit number, the date the plan was drawn or last revised, and the name of the person who prepared it.
- File format: PDF preferred. Hand-drawn plans are acceptable provided they are legible and labelled.

For a mobile site, you also need two floor plans, but they cover two different spaces:

1. A **vehicle floor plan** showing the inside of the drug-checking vehicle: where the workstation is, the client area (if your model lets clients inside), the surface where you prepare samples, and any onboard secure storage. 2. A **fixed-base storage office floor plan** for the room at your parent organization where samples wait overnight and between operating shifts. **Substances cannot stay in the vehicle outside operating hours** — see Section III.8 and Part IV for more on this.

> **REDACTED EXAMPLE**
>
> *A redacted approved fixed-site application on file includes a building-level layout showing how the drug-checking office sits inside the larger clinical hub. The detail plan of the drug-checking office shows the workstation, the locked cupboard holding both safes, and two panic buttons positioned out of client sight.*

III.4 Building or Vehicle Exterior Photograph

You include a photo of the building (for fixed sites) or the vehicle (for mobile sites) so Health Canada reviewers can picture the place. The photo is not part of your security — it is just for context. A clear, daylight, front-facing photo showing the entrance or the vehicle as it normally looks during operating hours is enough.

For **fixed sites**, the photo should clearly show:

- what the building looks like from the street
- where the entrance is

If your site is in a mixed-use building (for example, a community health centre that shares space with other tenants), an interior photo showing the drug-checking suite door from the corridor is helpful but not required.

For **mobile sites**, the photo should show:

- the vehicle from the side
- any program branding the public can see (for example, an outreach wrap)
- the deployment set-up if you use a privacy tent as part of your site perimeter (see Part IV.2)

- Fixed site: building exterior photo, daylight, front-facing, showing the entrance.
- Mobile site: vehicle exterior photo, side view, showing branding if applicable.
- Mobile site (if using privacy tent): photo of the deployed configuration with the tent set up alongside the vehicle, demonstrating the physical perimeter the tent forms.
- File format: JPG, PNG, or PDF. High enough resolution to be legible when printed.

III.5 Substance Custody and Courier Log Template

The Substance Custody and Courier Log records the chain of custody for every sample you send from your site to Western University for confirmation testing. Health Canada requires three things:

- the log must exist before any sample leaves your site
- the log must stay on site for as long as the exemption is valid
- the log will be reviewed during inspections to confirm the chain of custody described in your protocols is actually being followed

Western's network uses one standard log template. **Do not design your own.** Each line in the log is one sample (one Unique ID, one envelope, one entry). Every handover (client to staff, staff to safe, safe to courier, courier to Western) gets a signature. The log stays on site at all times.

- Entry Number — sequential identifier; the same number is also written on the sample envelope.
- Date and time the substance was submitted by the client.
- Location where the substance was submitted (drug-checking room within the Site, or specific stop for mobile).
- Name and signature of the staff person who packaged the sample and placed it in the secure storage safe.
- Date and time the sample was entered in the Scatr dashboard "laboratory tests" subsection (which signals to Scatr that a pickup is needed) and the Unique ID generated by the dashboard.
- Name and signature of the courier who collected the sample on pickup day.
- Verification that the courier's name matches the photocopied government-issued ID retained on file (the Site keeps a photocopy of every courier's ID for cross-reference).
- Date and time the sample was removed from the safe and handed to the courier.
- Name, signature, and initials of the witnessing RPIC or A/RPIC.
- Date the corresponding result PDF is received from Western (added later, when results return).

REDACTED EXAMPLE

The approved log template is a single-page Word document used as a master. Physical log books are printed from it. Sites usually keep a bound logbook on site (pages cannot be removed without leaving a mark) rather than loose pages.

*Under the Western model, four people hold keys to the courier lockbox: the receiving lab's Principal Investigator, the courier coordinator, one designated harm-reduction worker per partner site, and one more designated lab staff member. These four names go in your **protocols document**, not in the log itself.*

■ HC FLAG

In past reviews, Health Canada has flagged that the Sample Custody Log needs a **sample-donor signature field** — a place where the client signs to confirm they are giving the sample voluntarily and that they understand they will not get the sample back. Make sure your log template has this field, even if your client usually only signs a label on the envelope. Your protocols and your log template must agree.

III.6 Unknown Substances Left Behind Log Template

The Unknown Substances Left Behind Log records every time a client leaves a substance in the drug-checking area. This includes substances left on purpose (for disposal) and substances left by accident.

Health Canada requires this log because once a client leaves a substance behind, the site is now in possession of it. The exemption only covers possession inside the named site, and only for drug checking or disposal. So you need a written record of the path from "left behind" to "transferred to local police for destruction."

- Entry Number — sequential identifier; written on the envelope holding the substance.
- Date and time the substance was found.
- Location where the substance was found (specific room, surface, or — for mobile — specific stop).
- Name and signature of the staff person who found, sealed, and placed the substance in the locked safe.
- Date and time the local police service was contacted to request pickup (within 24 hours of finding the substance).
- Name, badge number, and signature of the police officer who collected the substance.
- Date and time the substance was removed from the Site.
- Name, signature, and initials of the witnessing RPIC or A/RPIC.

HC FLAG

The local police service named in your log must be the same service named in your protocols and the same service Health Canada writes into the Other Terms and Conditions of your decision letter. **All three documents must name the same service.** If your site sits on the boundary between two police services (rare), name the primary service and explain the boundary in your application.

III.7 Responsible Person In Charge (RPIC) Documentation

Health Canada wants three items of personal documentation for the RPIC. They also want a small amount of information about at least one Alternate Responsible Person in Charge (A/RPIC).

The RPIC does **not** appear on the Application Form at all. Section 2.1 names the Principal Investigator, who is a different person with a different role (see II.3). The RPIC is named in your protocols document and supporting package, and is personally responsible for following the rules in the exemption for the whole time the exemption is valid.

RPIC documentation (required)

- Resume or curriculum vitae covering the most recent ten years of employment, with locations. The location detail is what Health Canada uses to determine whether a foreign Criminal Record Check is also required (any period of residence outside Canada in the last ten years triggers a foreign CRC requirement for each country of residence).
- Valid Canadian Criminal Record Check (CRC) issued by a Canadian police service. The CRC must explicitly cover both Designated Criminal Offences (Criminal Code ss. 83.02–83.04, 380–382, 462.31, 467.11–467.13) and Designated Substance Offences (CDSA Part I, excluding s.4(1) possession). The CRC must be less than twelve months old at the time of submission (Health Canada's requirements list, October 15, 2026).
- Foreign CRC equivalent for each foreign country in which the RPIC has resided in the last ten years (if applicable). Some countries do not issue equivalents; in those cases the RPIC must provide a sworn declaration explaining the gap.

Health Canada's answers of September 17, 2026 also settle two things about these roles. There is no education requirement for the RPIC or the A/RPICs. And the RPIC, or in their absence an A/RPIC, must be present at the site at all times during operating hours: not necessarily in the drug checking space, but within the same building and on the same floor. Every decision letter in the network carries a condition to that effect, so the staffing section of the protocols must show that a named person is always there.

A/RPIC documentation (required for fixed sites; expanded for mobile)

- Name and position title for at least one A/RPIC. No resume or CRC required by Health Canada at the federal exemption stage.
- The list is maintained on Site, kept current, and made available during inspection.
- The role of the A/RPIC and the conditions under which the A/RPIC stands in for the RPIC must be described in the protocols document (Section 1.2 Staff Roles).

■ For mobile sites: a more comprehensive A/RPIC list is recommended because mobile units typically rotate two-person crews across multiple operating days, and at least one named A/RPIC must be present during every operating shift in addition to (or instead of) the RPIC.

■ HC FLAG

Use the same RPIC title in every document. Health Canada will flag any mismatch between the RPIC's title on the cover letter, the protocols document, the training log, and the chain-of-custody log. Note that the RPIC is not named on the Application Form: Sections 2.1 and 2.2 name the Principal Investigator, and that name must be consistent separately. Pick one title (for example, "Director, Health Services") and use it the same way every time. If the RPIC also has a clinical role (for example, M.D.), use the administrative title in the operational documents. The administrative title is the one that shows who is accountable for the exemption.

III.8 Mobile-Only Supporting Documents

Mobile applications need the seven documents above (Sections III.1 through III.7) **plus** the four mobile-specific documents below. Each one answers a question Health Canada is likely to ask during review (see Part IV.2).

III.8.1 Vehicle Floor Plan

A floor plan of the vehicle interior. Show:

- where the workstation is
- the client area (if your model lets clients inside the vehicle)
- the surfaces where you prepare samples
- any secure storage on board

This is on top of the fixed-base storage office floor plan covered in Section III.3.

III.8.2 Stop Addresses Document

A list of every regular stop the mobile unit makes. For each stop, include:

- the address
- the day(s) of the week the unit is there
- the time of day the unit is parked

For stops on **private property**, name the property owner and confirm the landlord supports the program (see Part IV.2). For stops on **public property**, name the municipal authority that supports the program. For stops in **confidential locations** (no private-property parking), describe the stop without giving a specific address. Health Canada accepts confidential stops as long as you describe the general geographic area.

III.8.3 Route Plans

A visual route map for each operating day. Each map should show:

- the order of the stops
- the route between stops
- where the shift starts and where it ends

A redacted approved mobile package on file includes three route plans, one for each weekly operating day.

- Tuesday route plan (or whichever day the mobile operates first in the week): start location, stop sequence, transit between stops, end-of-day return location.
- Wednesday route plan: same structure.
- Thursday route plan: same structure.
- Add additional plans for any other operating day.
- File format: PDF map screenshots (Google Maps, OpenStreetMap, or equivalent) annotated with stop numbers and times are acceptable.

III.8.4 Outreach and Client Notification Materials

Documents that show how clients find out the operating schedule and any changes to it.

The most common method is a **dedicated phone line** clients can call for the current schedule. The same line is also the back-up when the vehicle is out of service for repairs. During those down-times, the program lead picks up the line and offers harm-reduction information by phone during what would have been the operating shift.

Other outreach materials often included with mobile applications:

- postcards or flyers handed out at outreach events showing the weekly schedule
- social media or community-board posts announcing schedule changes
- a simple webpage listing days, times, and stops

> **REDACTED EXAMPLE**
>
> *A redacted approved mobile application on file includes two double-sided outreach postcards (drug-testing hours and locations on one side; program description and dedicated phone number on the other) and a community-facing photo of the deployed mobile unit.*

Part IV — Anticipated Health Canada Follow-Up Questions

Almost every Section 56(1) drug-checking application gets a written **Request for Information (RFI)** from Health Canada before a decision is made. This back-and-forth is normal. It is the main reason the real timeline runs to several months, even though Health Canada's published average is 70 calendar days.

The questions are very predictable. The same handful of topics come up over and over, across sites in different provinces with different operating models. If you address them in your initial submission, the back-and-forth gets much shorter.

The patterns below come from real Health Canada correspondence with partner sites in Western's network during 2024–2026 reviews. All site identifiers have been removed.

IV.1 Threshold Issues Health Canada Always Checks First

Health Canada checks the following items the moment they open your file, before they start a substantive review. If any one of these is wrong, your file stops there until you fix it.

- Site-specificity. One application equals one Site. A fixed brick-and-mortar location and a mobile outreach unit cannot be combined in a single application — they require two separate applications, each with its own decision letter and its own ongoing compliance obligations. Two fixed sites operated by the same organization at different addresses also require two applications. Health Canada will return a combined application without review and require it to be split.
- Co-location with a Supervised Consumption Site (SCS). Health Canada asks every drug-checking applicant whether the proposed Site is co-located with a licensed SCS. If yes, drug-checking inside that SCS may already be authorized under the SCS's existing s.56.1 exemption — verify before submitting a separate s.56(1) application.
- RPIC name and position title. Both fields must be specified explicitly; "to be determined" is not an acceptable answer.
- RPIC presence. The RPIC, or in their absence an A/RPIC, must be at the site at all times during operating hours (same building, same floor). Name at least one A/RPIC, and more if shifts are led by different people.

IV.2 Mobile-Specific RFI Questions

Mobile applications get the most follow-up questions. The most common one — and the most important one to get right — is the **Site perimeter question**: where exactly does your legally exempted site begin and end during a mobile operating shift?

The "client must be inside the perimeter" question

This is the most common reason a mobile application is sent back. If your model has the client standing outside the vehicle while staff do the test inside the vehicle, Health Canada will **not** issue the exemption as submitted.

The reasoning Health Canada has used in recent reviews:

- The CDSA's rules against possession (s.4(1)), production (s.7(1)), and transfer (s.5(1) and 5(2)) apply to the **client**, not just to staff.
- The Section 56(1) exemption only protects activities that happen **inside the named site**.

- If the client is outside the site perimeter when the substance is handled or passed to staff, the client is not protected by the exemption.
- That means the activity is not lawful as set up.

Health Canada has accepted two ways to fix this:

- Revise the model to allow clients inside the vehicle during testing. The vehicle interior is the Site perimeter. Clients enter, the test occurs, the client exits. This requires the vehicle to physically accommodate clients and to have a workstation configuration that permits a client to be present during testing.
- Revise the proposed Site to include a defined external perimeter. The standard arrangement is a three-sided privacy tent set up alongside the vehicle, with the tent forming the legally exempted Site for the duration of the visit. The tent perimeter must be described in the application; a photograph of the deployed configuration in Section III.4 is helpful.

The other recurring mobile RFI questions

- Vehicle parking address outside operating hours. Where is the vehicle parked when not in service? Substances cannot be stored in the vehicle outside operating hours, so this address is also where overnight sample storage occurs (in a fixed-base storage office on the parent organization's premises).
- Route plan and geographic range. The neighbourhoods and stops the unit will visit, or the geographic boundary within which it operates. Health Canada must be notified prior to any modification that takes the route outside the original operating district, and prior to any modification that takes the route outside the province.
- Client notification mechanism. How clients are informed of the route, stop arrival and departure times, and any schedule changes. A dedicated phone line for the program is the most common mechanism.
- Mechanical contingency plan. What happens when the vehicle is out of service for repair. The most common approach is for a designated manager to take the dedicated client phone line and provide harm-reduction information during what would have been the operating shift.
- Confirmation of no overnight substance storage. An explicit statement that illegal substances will not be stored inside the mobile unit outside operational hours.
- Landlord and municipal support. Confirmation that municipal authorities support the program for stops on public property; landlord support for any stops on private property. Stops that occur in confidential locations (without parking on private property) should be described as such.
- High-volume and overflow procedures. The minimum staffing model on the unit (typically at least two workers per shift), how wait times are communicated, and the policy for limiting samples per client during periods of high demand (typically a one-sample-per-client rule during overflow).
- Chain of custody from the vehicle to the fixed storage location (October 2026). A record of the transfer of substances from the mobile unit to the overnight storage site: sample IDs, the date of transport, and the name and signature of the staff member who carried it, either on the existing logs or

on a dedicated log, with the policy stating that the staff member who stored the substance in the vehicle is the one who transports it.

- Every stop with an address and permission (October 2026). A stop described only as a "community outreach location" drew a request for the address and confirmation that the property owner had given permission.

IV.3 Operational RFI Questions (Both Fixed and Mobile)

These questions come up for both fixed and mobile applications. Answering them in your initial submission is easy to do and shortens the review a lot.

- Days and hours of operation, including any seasonal or weekly schedule variation. Health Canada writes the operating schedule into the decision letter; significant changes after issuance trigger an operational change notification (see Part V.4).
- Sample data tracking from police submissions. If the Site accepts samples from police (an unusual but not impossible scenario), what data is captured and whether results are communicated back to the submitting officer.
- Result communication mechanism. How drug-check results are returned to whoever provided the sample (usually the client, via verbal communication on screen at the time of the scan), including the timing and format of the communication for any offsite results that arrive later.
- Multi-technology testing protocol. If samples will be tested on Site with more than one technology — for example, the Scatr Series One Raman/NIR spectrometer plus immunoassay strips — the protocol must describe how the technologies are used together, the sequence in which they are run, and how conflicting results are interpreted.
- Mass spectrometry handoff. If samples will be sent to Western for offsite mass spectrometry, the protocol must describe what happens to remaining sample material after testing (Western retains residual material for the duration of HSREB record-retention obligations and disposes of it under its own exemption's destruction protocol).
- Sample preparation and analysis protocol. Detailed step-by-step description of how a sample is prepared for analysis and how the analysis is performed. The redacted approved partner-site protocol (Section 1.5 of the protocols document) lays this out as a 17-step procedure; HC accepts that level of detail.
- Chain-of-custody logs for samples stored on Site. The Substance Custody Log (Section III.5 above) covers samples awaiting offsite testing. If samples are retained on Site after testing for any reason, the protocol must describe that retention separately.
- Sample Custody Log signature field. The log must include a sample-donor signature field, and the log description in the protocols document must align with the actual log template.
- Facility and Sample Pickup protocol. The protocol covering courier pickup of samples must explicitly reference completion of the Substance Custody Log entries — naming the document and the entries it requires.

- Offsite testing preparation protocol. Detailed description of how samples are prepared for shipment to a permitted recipient (Western, in the SUAP context).
- Room number. Whether the drug checking room has a room number, and whether every document refers to the same room (March 2026).
- Storage container type. Whether the secure storage is a safe, a lockbox or a cabinet; "safe, lockbox, or cabinet" in one document drew a request to say which (October 2026).
- Floor plan features on the plan. All access-controlled doors, the client flow out of the building, any intercom used to admit clients, and the safe for substances left behind must be drawn on the plan itself, not described in text beside it (March and October 2026).
- Test strips. Whether test strips will be offered on site; if so, a policy covering how they are used, the data recorded, and how residue and used strips are disposed of. Health Canada treats a test strip as different from a registered test kit (March 2026).
- Which form records which transfer. Each form must say whether it documents transfer for off-site testing, transfer to the police, or both (October 2026).
- Forms named in the policies. Any checklist or form that must be completed before a shipment leaves, or when a substance is left behind, must be named (or linked) in the policy that staff follow (October 2026).
- Consistent data fields. The demographic fields listed in the application must match the intake form; age missing from one and present on the other drew a request to reconcile them (October 2026).

IV.4 RPIC and Staff RFI Questions

- RPIC ten-year employment history with locations. The location detail is what Health Canada uses to determine whether a foreign Criminal Record Check is required. Any period of residence outside Canada in the last ten years triggers a foreign CRC requirement for each country of residence. Any gap of greater than ten years in Canada (e.g., a return after a long period abroad) requires explanation including foreign-residence dates and country.
- RPIC title consistency. The RPIC's title must be identical across the cover letter, the protocols document, the training log, and the chain-of-custody log. The Principal Investigator's name and title must likewise be consistent between the Application Form (Sections 2.1 and 2.2) and the cover letter. Do not conflate the two roles: inconsistencies in either generate an automatic RFI.
- A/RPIC list. At least one A/RPIC must be named, and the protocols document must describe the role and the conditions under which an A/RPIC stands in for the RPIC.
- Training log structure. The training log must record the trainee's name and identify who is responsible for delivering training (named individual, not just role title).
- Key and code distribution. The job title of the person responsible for distributing keys and combinations, when they are distributed, and confirmation that records of distribution are kept (March 2026).

- Community notification before the decision. The public must be notified of the proposed service before a decision is made: how, on what timeline, and which stakeholders; and whether the local police service has been consulted and what it said (March 2026). Health Canada's requirements list (Appendix I, section A) describes the engagement it expects.

IV.5 Lost or Stolen Substances and Destruction RFI Questions

- Lost or Stolen Substances Procedure. Must explicitly state that the completed Health Canada Loss or Theft Report Form will be sent to the Office of Controlled Substances within 24 hours via exemption@hc-sc.gc.ca, in addition to the local police service notification. Three destinations within 24 hours; one log per substance (not per incident).

- Transportation loss procedure. Must specify what staff will do if a sample is discovered lost or stolen during transit — including notification of the courier coordinator, the receiving lab at Western, and Health Canada.

- Sample destruction log. Must include the verbatim statement that "the controlled substance destroyed has been altered or denatured to such an extent that its consumption has been rendered impossible or improbable." This exact phrasing is required.

- Destruction witness requirement. The witness must include either the RPIC or an A/RPIC; a peer worker alone is not sufficient.

- Substance Disposal protocol. Must include the name of the record-keeping document, the items it tracks, confirmation that destruction is witnessed by a secondary staff member, and the requirement that the RPIC or an A/RPIC is one of the staff involved in the destruction.

- Substances left behind form fields. The form must carry the sample identification code, the name of the staff member who packaged and stored the substance, the date the police were contacted (within 24 hours), the storage location, and the receiving officer's name without an "if known" qualifier (October 2026). The full field list is in Appendix I, section E.

- Loss or theft policy scope. Wording borrowed from pharmacy or licensed producer settings (references to pharmacists, licensed producers, Indigenous Services Canada, or administrative fields the Health Canada form does not ask for) drew a request to revise (October 2026). Keep the policy to what the exemption covers and to the fields on the Health Canada form.

IV.6 The "Possession, Production, Transfer" Framing

Health Canada looks at three named activities when reviewing a drug-checking application. Your protocols document needs to cover all three. The Section 56(1) exemption protects the named people (RPIC, A/RPIC, designated staff, clients) when they do these activities — but **only inside the named site perimeter, and only for drug checking or disposal**.

- Possession (CDSA s.4(1)). Occurs when a client or a staff member is handling an illegal substance during drug checking. Both client and staff must be inside the Site perimeter when this occurs, which is why the perimeter question (Part IV.2 above) is so consequential for mobile applications.

- Production (CDSA s.7(1)). Occurs when a small number of solid granules are dissolved into a liquid solution, because dissolution changes the physical state of the substance. If your testing methodology involves any dissolution step (immunoassay strips typically do; Raman scanning typically does not), production is occurring on Site and must be authorized under the exemption.
- Transfer (CDSA s.5(1) and 5(2)). Occurs when a client passes an illegal substance to a staff member for drug checking, or when a staff member disposes of a substance left behind by a client by handing it to law enforcement. Both transfer events must occur within the Site perimeter; both must be described in the protocols.

This is also why the substances-left-behind protocol matters so much to Health Canada:

- A substance accidentally left in the drug-checking area is **possession** by the site.
- The police pickup is a **transfer**.
- Both events must be written down in the Unknown Substances Left Behind Log (Section III.6) and traced back to your protocols (Section 1.7 of the operational protocols document).

Part V — After Approval: Ongoing Compliance Obligations

A Section 56(1) drug-checking exemption comes with a set of terms and conditions you must follow. Breaking them can lead to suspension or revocation of your exemption.

Decision letters are organized by named sections: Definitions, Scope, Suspension Without Notice, Revocation, Duration, Other Terms and Conditions, and Reporting. Inside the **Other Terms and Conditions** section is a numbered list of operating requirements. The number of items varies by site type and scope.

The most important ongoing duties are summarized below. Always check the exact wording of your own decision letter.

V.1 Monthly Reports

You must send a monthly report to exemption@hc-sc.gc.ca by the **15th of each month**. The report must cover six things:

- The number of drug checks performed at the Site during the reporting month.
- The general demographics of clients served, typically aggregated age and gender (no individual identifiers).
- The results of the drug checking, including whether substances identified were as expected or inconclusive.
- Whether the result influenced the client's decision to retain or discard the illegal substance.

- The number of illegal substance samples sent offsite for further testing (to Western or another permitted recipient).
- If known, the results of the offsite drug checks.

For SUAP partner sites, the Scatr Series One device produces a downloadable monthly report that already contains most of these items. Add or update anything that is missing (for example, offsite results that have come back, or a summary of how results affected client decisions). Send the combined report to the Office of Controlled Substances by the 15th.

> **HC FLAG**
>
> Late or missing monthly reports are the most common compliance issue Health Canada flags during inspections. Set a recurring calendar reminder for the first week of every month. If your report is going to be late for any reason, email exemption@hc-sc.gc.ca ahead of time. Do not miss the deadline silently.

V.2 Loss or Theft Notifications (within 24 hours)

Any loss or theft of a controlled substance must be reported **within 24 hours of discovery**. This applies whether the loss or theft happens at your site or in transit between your site and Western. You need to send three notifications:

- The local police service named in your decision letter and protocols.
- The Office of Controlled Substances at exemption@hc-sc.gc.ca.
- The Health Canada Loss or Theft Report Form, completed in entirety and submitted by email to the same OCS address. The current form is available at the Health Canada website under "controlled substances compliance monitoring".

Fill in **one log sheet per substance**, not one per incident. If three samples are lost in one event, fill in three log sheets. Keep all records on site for the whole time the exemption is valid.

V.3 Site Closure or Pause Notifications (within 48 hours)

If your site stops offering drug-checking services, you must tell the Office of Controlled Substances **within 48 hours**. This applies no matter why the closure happens. Examples include:

- you are voluntarily winding down the program
- you unexpectedly lose your premises
- the RPIC leaves and there is no immediate replacement
- the mobile unit is out of service for more than 48 hours

V.4 Operational Change Notifications (within 10 working days)

Any change to your site's security, physical layout, or supporting resources means you must notify Health Canada **within 10 working days** of the change taking effect. You also need to send updated policies and procedures along with the notification.

- Change of RPIC: notify within 10 working days; the new RPIC cannot work at the Site until Health Canada has the new RPIC's CRC on file.
- Change of A/RPIC: notify and update the on-Site list; no CRC required.
- Change of physical layout (room reconfiguration, addition of cameras, change of safe location): notify with revised floor plan.
- Change of security elements (new locks, change of keyholders, new alarm system): notify with revised security policy.
- Change of operational hours (significant change, not week-to-week variation already described in the application): notify with revised hours-of-operation document.
- Change of waste vendor or disposal pathway: notify with revised disposal protocol.
- Change of police service jurisdiction (rare): notify with revised left-behind protocol.

V.5 Renewals

Apply to renew before your current exemption expires. Each renewal gets a **new HC file number** (format YY-NNNNNN-NNN), but the underlying **HC6 reference** (long-form, like HC6-53-144-NN) stays the same across renewals. The HC6 reference is the stable number to track over time and to use in correspondence.

In practice, partner sites in Western's network often receive **one-year renewal terms**, even though Health Canada's general scientific-purposes page lists two years. Plan to start putting your renewal package together about **two months before** your expiry date. That gives you time for the back-and-forth Request for Information loop.

V.6 Records Retention

You must keep all records required by the exemption on site for the whole time the exemption is valid. Health Canada can ask to see them at any time:

- Training logs (every staff member, every required training topic, dated and signed).
- Security logs (key/card distribution, lost-credential responses, alarm activations).
- Substance Custody Logs (every sample sent to Western).
- Unknown Substances Left Behind Logs.
- Destruction Logs (when applicable).
- Loss or Theft Report logs and submitted forms.

- All policies and procedures (including version history when revised).

The minimum time you must keep records is the length of the exemption. Many sites keep records for **two extra years after the exemption expires**, just to be safe. Some records (for example, training records tied to a named staff member) may need to be kept even longer under provincial occupational-health rules.

V.7 Inspection Access

Health Canada inspectors can visit your site at any time during the exemption term to check that you are following the rules. Most inspections are arranged ahead of time, but Health Canada can also do an **unannounced inspection** if there is a reason. Make sure your on-site records (logs, training records, policies) are organized and easy to find on short notice.

V.8 Anatomy of an Approved Decision Letter

A Health Canada decision letter approving a Section 56(1) drug-checking exemption follows the same general structure every time. Knowing what to expect helps you check, when you receive your letter, that everything you need is in there.

- Header: file number (format YY-NNNNNN-NNN, where YY is the year of filing); HC6 reference (format HC6-53-144-NN for drug-checking exemptions, where the trailing NN is a site-sequential counter preserved across renewals); date of issuance; addressee (the named exemption holder at their address).
- Definitions: approximately fifteen defined terms applicable to the exemption. The standard list includes Alternate Responsible Person In Charge, Client, Designated Criminal Offence (with Criminal Code section references), Designated Substance Offence (with CDSA Part I reference), Drug Checking, Health Canada Laboratory, Illegal Substance, Licensed Dealer, Minister, OCS, Responsible Person In Charge, Section 56.1 Exemption Holder, Site (with the precise physical perimeter), Staff Member, and Subsection 56(1) Exemption Holder.
- Scope: the CDSA provisions (typically subsections 4(1), 5(1), 5(2), and 7(1) of the CDSA, plus 6(1) and 6(2) of the Precursor Control Regulations) from which the named classes of persons are exempted; the activities authorized; the physical boundaries within which the exemption applies.
- Suspension Without Notice and Revocation clauses: standard regulatory text affirming the Minister's discretionary authority.
- Duration: typically one year in practice for partner sites. The exemption expires on the earliest of the calendar date specified, the date of revocation, or the date superseded by a new exemption.
- Other Terms and Conditions: a numbered list of operational requirements. The list length varies by site type and scope; verify the exact count and wording in your own decision letter. Common topical clusters: training and oversight; substance-handling restrictions; substances left behind by clients; police disposal pathway; physical security; operational change notifications; criminal record checks for the RPIC; community engagement; data sharing and records access; and the monthly reporting requirement.

- Closing: the signing officer's name and title (Director or Acting Director, Office of Controlled Substances), and a closing paragraph noting that Health Canada welcomes pertinent information throughout the validity period.

> **REDACTED EXAMPLE**
>
> *Recent decision letters in Western's network have been signed by the Director of the Office of Controlled Substances. The person who signs may change as staff change. The **title** (Director or Acting Director, OCS) is what matters.*

Part VI — Reference Appendices

Appendix A — Health Canada Contacts

Contact	Role	Email	Phone
Office of Controlled Substances — Exemptions Division	New applications, renewals, monthly reports, loss/theft notifications, all correspondence	exemption@hc-sc.gc.ca	343-990-4862 (given in HC correspondence, March 2026)
Health Canada Loss or Theft Reporting	Loss/theft of controlled substances or precursors	exemption@hc-sc.gc.ca + the official PDF form (Appendix G)	not publicly listed
Government of Canada ATIP Online Request Service	Formal Access to Information and Privacy requests	atip-aiprp.apps.gc.ca (online portal)	not applicable
Prof. François Lagugné-Labarthet	Western SUAP Principal Investigator	flagugne@uwo.ca	519-661-2111 x81006
Cameron Brown	Western SUAP Project Manager	cbrown58@uwo.ca	226-238-9970

Mailing address (only for paper documents that need a wet signature): Exemptions Division, Office of Controlled Substances, Health Canada, 161 Goldenrod Drwy, AL0300B, Ottawa ON K1A 0K9. (Format taken from Health Canada's current published Section 56(1) scientific-purposes form page, verified November 2026.)

Appendix B — Glossary of Health Canada-Defined Terms

The definitions below are taken word-for-word from recent Health Canada decision letters issued to drug-checking sites in Western's network. These are the active definitions used in the Other Terms and Conditions sections of those letters. Use the same terms in your protocols document so your wording matches Health Canada's.

Term	Definition (from Health Canada decision letters)
Alternate Responsible Person In Charge (A/RPIC)	A person responsible, when at the Site, for performing drug-checking services and, in the RPIC's absence, for ensuring compliance with the exemption's terms and conditions.
Client	An individual who is at the Site to have illegal substances tested.
Designated Criminal Offence	An offence involving terrorism financing (Criminal Code ss. 83.02-83.04), fraud (ss. 380-382), laundering proceeds of crime (s. 462.31), or criminal organization activity (ss. 467.11-467.13), or any conspiracy or attempt in relation to such offences.
Designated Substance Offence	An offence under Part I of the CDSA other than subsection 4(1), or any conspiracy or attempt in relation to such an offence.
Drug Checking	A service where substances, which may be illegal substances, are tested at the Site or offsite by a Health Canada laboratory, licensed dealer, s.56.1 exemption holder, or s.56(1) exemption holder, to determine their purity and/or content.
Health Canada Laboratory	Any analytical laboratory operated by Health Canada.
Illegal Substance	A controlled substance or precursor that is obtained in a manner not authorized under the CDSA.
Licensed Dealer	The holder of a valid controlled-substances dealer's licence issued under the Narcotic Control Regulations, the Benzodiazepines and Other Targeted Substances Regulations, Part G of the Food and Drug Regulations, or Part J of the Food and Drug Regulations.
Minister	The federal Minister of Health.
OCS	The Office of Controlled Substances, Controlled Substances and Overdose Response Directorate, Health Canada.
Responsible Person In Charge (RPIC)	The person designated by the applicant who is responsible, when at the Site, for the operational oversight of drug-checking services and for ensuring compliance with the exemption's terms and conditions.
Section 56.1 Exemption Holder	The holder of a valid Section 56.1 exemption authorizing activities, including drug checking, at a supervised consumption site.

Term	Definition (from Health Canada decision letters)
Site	The defined physical premises within which the exempt activities are authorized. For fixed sites, a specific room or set of rooms within a building. For mobile sites, the interior of the vehicle (and any extended perimeter such as a privacy tent).
Staff Member	An individual employed by or under contract with the applicant organization, or with a partner organization (such as Scatr or Western), to perform drug-checking services at the Site.
Subsection 56(1) Exemption Holder	The holder of a valid Subsection 56(1) exemption authorizing drug-checking activities involving illegal substances.

Appendix C — Pre-Application Preparation Checklist

Work through this checklist **before** you open the Health Canada application form. Settling each item ahead of time makes filling out the form faster and reduces the number of follow-up questions Health Canada is likely to ask.

- Decided whether to file one application or two (separate applications required for fixed and mobile).
- Identified and confirmed RPIC: name, position title, willingness to serve.
- Obtained valid Canadian Criminal Record Check for RPIC, less than twelve months old on the submission date.
- Obtained foreign CRC equivalent for any country in which the RPIC has resided in the past ten years.
- Compiled RPIC ten-year employment history with locations.
- Identified at least one Alternate RPIC (name, position title).
- Defined the Site physical perimeter (and the privacy-tent perimeter for mobile sites where clients will not enter the vehicle).
- Identified the storage location for samples awaiting offsite testing and the security elements protecting it.
- Identified the disposal pathway for substances destroyed on Site (and the waste vendor, if applicable).
- Confirmed that Western University is the named permitted recipient for offsite mass spectrometry samples.
- Confirmed the exact name of the local police service that will receive 24-hour notifications and pick up substances designated for police disposal.
- Drafted the operational protocols document covering all required topics (Section III.1).

- Obtained the Letter of Information and Consent template from Western under the HSREB-approved study protocol (Section III.2).
- Prepared site floor plans (Section III.3) and exterior photograph (Section III.4).
- Confirmed the monthly reporting workflow (responsible person, calendar reminder, submission email).

Appendix D — Application Package Contents Checklist (Fixed Site)

Use this checklist to put together the full submission package for a fixed-site application. Every item should be present and labelled before you email the package to exemption@hc-sc.gc.ca.

- Cover letter (one page) introducing the applicant organization, naming the proposed RPIC and A/RPIC, identifying the proposed Site, stating that this is a new s.56(1) drug-checking exemption application, and listing all attached documents.
- Completed Health Canada Application Form (10 pages) with Sections 1, 2, 3, 6, and 7 filled in; Section 4, 4.1, 5 left blank; Application Checklist on page 10 with Sections 1, 2, 3, 6, 7 checked.
- Document A: Operational protocols document (Sections 1.1 through 1.8 per Section III.1 of this guide).
- Document B: Letter of Information and Consent (Western HSREB template).
- Document C: Site floor plan(s) — building-level overview and drug-checking room detail.
- Document D: Building exterior photograph.
- Document E: Substance Custody and Courier Log template.
- Document F: Unknown Substances Left Behind Log template.
- Document G: RPIC documentation — ten-year employment history, valid Canadian CRC, foreign CRC if applicable; A/RPIC name and position.

Appendix E — Application Package Contents Checklist (Mobile Site — Additional Items)

Mobile applications include everything in Appendix D **plus** the mobile-specific documents below.

- Vehicle floor plan (Section III.8.1) showing the workstation, client area (if clients enter the vehicle), substance-preparation surface, and any onboard storage.
- Fixed-base storage office floor plan (covered in Section III.3) showing where samples are stored overnight and between operating shifts.
- Stop addresses document (Section III.8.2) listing every regular stop with day-of-week and time-of-day windows.
- Route plans (Section III.8.3) for every operating day.

- Outreach and client notification materials (Section III.8.4) showing how clients learn the schedule, including the dedicated phone number.
- Vehicle exterior photograph (Section III.4) showing the vehicle as deployed; if a privacy tent is used as the Site perimeter, an additional photograph showing the deployed configuration.
- Expanded A/RPIC list (Section III.7) covering every A/RPIC who may staff the unit during operating shifts.
- Explicit confirmation in the protocols document that illegal substances will not be stored in the vehicle outside operational hours.
- Landlord and municipal support confirmations for stops on private and public property respectively (per Section IV.2).

Appendix F — Post-Approval Compliance Checklist

Use this checklist to confirm you are following the terms and conditions of an issued Section 56(1) drug-checking exemption.

- Monthly report submitted to exemption@hc-sc.gc.ca by the 15th of every month, with all six required elements.
- Loss or theft incidents reported within 24 hours to all three destinations: local police, OCS at exemption@hc-sc.gc.ca, and the Health Canada Loss or Theft Report Form.
- Site closures or pauses (any cause) reported to OCS within 48 hours.
- Operational changes (RPIC, security, layout, hours, vendor, jurisdiction) reported within 10 working days, with revised policies attached.
- Mobile route changes that take operations outside the original district reported in advance.
- Mobile route changes that take operations outside the province reported in advance.
- All training, security, custody, left-behind, destruction, and loss/theft records maintained on Site for the duration of the exemption (common practice is to retain for two years beyond expiry).
- Renewal application package assembled and submitted before the current exemption's expiry date.
- New RPIC's CRC submitted and confirmed on file with Health Canada before the new RPIC begins working at the Site.
- A/RPIC list kept current and available on Site.

Appendix G — Source Documents and References

This guide is built from the source materials below. If you are preparing an application, request access to any of these documents from the Project Manager as you need them.

Health Canada Official Forms and Guidance

- Application form for an exemption to use a controlled substance for scientific purposes (Opioid Response Team, Office of Controlled Substances, April 2018) — the official 10-page fillable PDF every applicant completes. Held on file with the project; also available from the Health Canada website.
- Canada Gazette Part II publication of the Controlled Substances Regulations (December 17, 2025; in force October 1, 2026). The subsection 56(1) route for drug checking continued after that date (Part I.4).
- Health Canada Loss or Theft Report Form — available at the Health Canada website under controlled-substances compliance monitoring.
- Directive on Physical Security Requirements for Controlled Substances — referenced in Section 6 of the application form. Published on Health Canada's Physical Security Directive page (written for licensed dealers; eleven security levels; page last modified November 30, 2018). URL in the list below.
- Controlled Drugs and Substances Act (CDSA) and Precursor Control Regulations — the underlying federal legislation; current consolidated versions available at justice.gc.ca.

Western SUAP Project Documents (held on file)

The following project documents are held on file at Western and inform the guidance in this document. None of the underlying partner-site identifiers are reproduced anywhere in this guide. Specific source documents are available on request from the Project Manager, subject to the partner site's consent.

- Approved partner-site Section 56(1) drug-checking exemption application packages (fixed-site and mobile-site examples) — the redacted basis for the field-by-field examples in Parts II and III of this guide. All site identifiers removed.
- Health Canada Exemptions Division, requirements list for drug checking site applications, sent to the project on October 15, 2026 (reproduced in Appendix I), and its answers to a partner site's pre-application questions, September 17, 2026 (no education requirement; co-applicants; RPIC presence; exemptions are location specific).
- Health Canada's written questions and recommendations to partner sites, March 2026 and October 2026 — the source of the items marked with those dates in Part IV.
- Western University HSREB-approved study protocol and Letter of Information and Consent template — used by every active SUAP partner site under Section III.2 (full LOI text reproduced verbatim in Appendix H).
- Partner-site Health Canada follow-up question correspondence — the source for the recurring HC follow-up question patterns described in Part IV. All site identifiers removed.
- Western University Drug-Checking Site Policies and Procedures — the upstream policies that inform the partner-site protocols template (Section III.1).

- Western University's own approved s.56(1) drug-checking exemption decision letter — reference for the operative HC-defined terms in Appendix B.
- A library of approved partner-site decision letters held on file at Western — cross-reference for terms-and-conditions patterns. Site identifiers redacted; access on request from the Project Manager.

Project Contact for Document Requests

For access to any of the documents listed above, contact the Project Manager: Cameron Brown, cbrown58@uwo.ca.

Verified Live URLs (Health Canada and Department of Justice)

The following URLs were fetched live and verified to return current published content as of November 2026.

- Health Canada — Section 56(1) exemptions landing page — https://www.canada.ca/en/health-canada/services/health-concerns/controlled-substances-precursor-chemicals/exemptions.html
- Health Canada — Section 56(1) scientific-purposes exemption application page (form, mailing address, processing time, validity term) — https://www.canada.ca/en/health-canada/services/health-concerns/controlled-substances-precursor-chemicals/exemptions/application-form-exemption-use-controlled-substance-scientific-purposes.html
- Health Canada — Physical Security Directive (Directive on Physical Security Requirements for Controlled Substances) — https://www.canada.ca/en/health-canada/services/health-concerns/controlled-substances-precursor-chemicals/controlled-substances/physical-security-directive.html
- Health Canada — Loss or Theft of Controlled Substances and Precursors (compliance page) — https://www.canada.ca/en/health-canada/services/health-concerns/controlled-substances-precursor-chemicals/controlled-substances/compliance-monitoring/loss-theft.html
- Health Canada — Loss or Theft Report Form (PDF) — https://www.canada.ca/content/dam/hc-sc/documents/services/publications/healthy-living/loss-theft-controlled-substances-precursors/loss-theft-report-form-controlled-substances-precursors.pdf
- Health Canada — Loss or Theft reporting guidance — https://www.canada.ca/en/health-canada/services/publications/healthy-living/loss-theft-controlled-substances-precursors.html
- Department of Justice — Controlled Drugs and Substances Act, Section 56 — https://laws-lois.justice.gc.ca/eng/acts/c-38.8/section-56.html
- Department of Justice — Controlled Drugs and Substances Act (full Act) — https://laws-lois.justice.gc.ca/eng/acts/c-38.8/
- Canada Gazette Part 2 — Controlled Substances Regulations (SOR/2025-242), published December 17, 2025 — https://gazette.gc.ca/rp-pr/p2/2025/2025-12-17/html/sor-dors242-eng.html
- Department of Justice — Controlled Substances Regulations (SOR/2025-242) consolidated text — https://laws-lois.justice.gc.ca/eng/regulations/SOR-2025-242/index.html

Appendix H — Letter of Information and Consent (Verbatim Template)

This is the current Western HSREB-approved Letter of Information and Consent (version 3, Version Date: October 7, 2026, approved by the HSREB on October 13, 2026). It is reproduced word-for-word from the source PDF on file (HSREB. LOIC_Device Use_V3_Oct 07 2026.pdf). Use the text below as your reference. For the formatted PDF version with the Western letterhead, request the source document from the Project Manager.

The LOI text below is reproduced verbatim and is not simplified or paraphrased. It is approved consent text under Western's HSREB study protocol and must be used as written.

Letter of Information and Consent

Project Title: Creating a Drug Checking Network Using Machine Learning Enabled Spectrometers

Principal Investigator (PI): Professor Francois Lagugne-Labarthet, Faculty of Science, Western University, London, ON, Canada. 519-661-2111 x81006 — flagugne@uwo.ca

Dear Prospective Participant,

We are inviting you to participate in a research study conducted by Dr. Francois Lagugne-Labarthet and Dr. Abe Oudshoorn of Western University. This project will provide voluntary drug-testing to clients of healthcare organizations across Canada who offer such services. Participants must be 18 years of age or older and have 0.5mg of substance or greater for testing.

What is the purpose of the study?

The purpose of this study is to give Canadians data regarding drug consumption. Currently, there is very limited access to drug-checking across Canada for people who use drugs (PWUD). The lack of drug-checking services leads to people consuming substances that they do not intend to consume such as fentanyl and its analogs. This leads to accidental overdoses and deaths. We want to understand both what drugs are being consumed in Canada, but more importantly, how learning about what is in your sample might impact decisions you make around using. This is currently taking place at more than 20 sites across Canada, and we will conduct 24,000-65,000 drug tests.

What will I do?

Your sample test will take either 5 minutes (short scan) or 15 minutes (long scan). The length of the scan means basically how detailed a view we take of your sample. This effects how precise we can be in terms of understanding exactly what is in your sample and in what amount. (In technical terms, the scan is using spectroscopy and a short scan uses 625 spectra whereas a long scan uses 2500 spectra.) While you wait for your drug test result we will ask you some demographic questions. It's your choice to answer these or not. When the result comes back, you can learn about what is in your sample and then we will ask if you will still be using this sample today, delaying use, reducing use, or choosing not to use the substance. Please note that the results are not 100% guaranteed and there is a small possibility of false positives or false negatives and the findings are not necessarily conclusive. If not using the substance, we will request your permission to send this to Western University in London Ontario for detailed analysis. The results of this analysis will be returned here for you to pick up if you wish. Western University may also send part of some samples on to Health Canada's Drug Analysis Service for extra confirmation testing. Choosing to participate or not today will not affect your access to other services here at the site.

What are the risks and benefits of the study?

There are risks to anonymity associated with the study. This includes the possibility that a worker unintentionally records identifying information about you. In order to prevent this, the device will delete a scan from the database if it contains a name, email, phone number, or address.

Secondly, we acknowledge that the information provided from the scan can put you in a tough situation to make choices about your use. Your harm reduction worker can't counsel you regarding what is in your test, but can provide you information about substances and substance use.

A final risk is that you may overdose after participating in the drug checking. Whether or not a controlled substance is detected by the device, your results will include a statement clarifying that the risk of consuming an unknown sample can result in adverse side effects, overdose, or death.

Can I withdraw from the study?

Yes, you may withdraw from the study at any time prior to publication of the findings without negative consequences. If you would like to completely remove your data from the study, you can do so by asking a site worker who will forward this request to the lead researcher for deletion of your data.

Compensation

There is no compensation for participation in this study.

Is the study voluntary and confidential?

Participation in this study is completely voluntary. The research team will strive to maintain privacy and confidentiality. You will not be required to provide any directly identifiable information such as your name or date of birth. With your permission, we will record your gender, age, racial/ethnic identity, preferred language, and if you identify as LGBTQ or another minority sexual orientation or gender identity. These will only be reported all together for all participants at a site, not individually.

Western University Health Sciences Research Ethics Board and representatives may access study materials for monitoring purposes.

Your data from the scan device database will be provided electronically to the Western researchers named above for analysis. This research data will be stored for 7 years post-publication, and subsequently deleted.

Results of the Study

Results of the scans will be shared monthly in a report to each site. The results of the study may be published in scholarly journals, presented at research conferences, and shared with relevant stakeholders in the health sector. The results will also be shared in a report the project funder, Health Canada.

Conflict of Interest

The testing devices are created by a company called Scatr. The study researchers have no conflict of interest in the work and no financial relationship to Scatr. Scatr has no on-site involvement in regard to data collection. All will be done by the site workers who have no conflict of interest. Additionally, Scatr is not doing the data analysis or generating the site reports. Data is simply being provided to the research team who will do the analysis and dissemination of results independently.

As the developer of the drug testing device, Scatr may financially benefit from the results of the study.

Consent

Providing your sample for testing is indication of your consent to participate.

Dr. Lagugne-Labarthet and/or others intend to claim sole ownership of any research results consistent with this consent. By consenting to this study, you agree that Dr. Lagugne-Labarthet can apply for patents and you will not receive any financial benefit that might come from the research.

For More Information:

Western University Health Sciences Research Ethics Board (HSREB) requires access to the study records to monitor the conduct of this research. Please contact the Office of Human Research ethics if you have questions or concerns about your rights as a participant and/or the conduct of this study.

Phone: (519) 661-3036 Email: ethics@uwo.ca

Please contact Dr. Lagugne-Labarthet, the Principal Investigator, with any questions pertaining to the study, including the purpose and participant requirements.

Phone: 519-661-2111 x81006 Email: flagugne@uwo.ca

Sincerely,

Professor Francois Lagugne-Labarthet Western University flagugne@uwo.ca

Version Date: October 7, 2026

Appendix I — Health Canada's Requirements List for Drug Checking Site Applications (October 15, 2026)

The Exemptions Division of the Office of Controlled Substances sent the project this list on October 15, 2026, in answer to a request for its in-depth list of requirements for a drug checking site application. It is reproduced here in Health Canada's order and with its section headings, with each item's wording kept as close to the original as the table allows. The right-hand column points to the part of this guide that deals with the item. Use the list as the final check before a package is sent: every item should be answered somewhere in the package, and the cover letter should say where.

A. General information

Item	Health Canada asks for	Where in this guide
A1	The name of the proposed drug checking site.	II.5 (Section 2.3), II.8 (Section 3), III.1 (title page)
A2	The exact location of the proposed drug checking service within the building: room name or number, floor, and address. For a service offered from a mobile vehicle, address items G2 and G3 instead.	II.5, III.3
A3	Applicant information: name, job title, name of the organization, address of the organization (which may differ from the address of the proposed service), and contact information.	II.3, II.4, cover letter (Appendix D)
A4	A brief description of the proposed drug checking service and its purpose.	II.8 (Section 3 further information), III.1 (1.1)
A5	The operating hours of the proposed service.	III.1 (1.1(c))
A6	For client-facing sites: a description of the community engagement completed, or to be completed before a decision, including all feedback received and the actions taken to address concerns; the measures that will collect ongoing feedback from the local neighbourhood once the service is open; and outreach to organizations such as nearby schools, childcare providers, business associations, local police and other local community groups. Examples include a community advisory committee, a publicly posted email address or phone number, ongoing consultations, or other outreach. Concerns must be documented and, where appropriate, mitigated.	IV.4 (community notification), Appendix D (community engagement summary)
A7	Confirmation of whether the proposed service is within 200 m of any school or daycare; if yes, their names and how the site will engage with them on an ongoing basis. Mobile services address item G8 instead.	Appendix D (community engagement summary)

B. Staffing

Health Canada's description: the Responsible Person in Charge (RPIC) is responsible for operational oversight of the service and is physically present at the site to supervise or perform drug checking. The applicant (A3) and the RPIC (B1) can be different individuals. When the RPIC is not at the site during business hours, an alternate RPIC (A/RPIC) may assume the RPIC's responsibilities, and there can be more than one A/RPIC.

Item	Health Canada asks for	Where in this guide
B1	The RPIC's name, job title and contact information.	III.7
B2	A criminal record check for the proposed RPIC that is less than 12 months old at the time of submission.	III.7
B3	A resume for the RPIC giving a timeline of employment experience, education and qualifications covering the past 10 years. The resume is used to determine whether a foreign criminal record check is also required.	III.7, IV.4
B4	A description of the staffing model, including the minimum number of staff per shift and the responsibilities of staff members.	III.1 (1.1(b), 1.2)
B5	A description of the training requirements for drug checking staff, and how the training will be documented.	III.1 (1.2(d)), IV.4

C. Security

Item	Health Canada asks for	Where in this guide
C1	The security measures in place at the site (keys, fobs, access cards or other measures), including who would have access to the room where drug checking occurs.	II.13, III.1 (1.2(b))
C2	The distribution of security elements: the name or job title of the person responsible for distributing them, confirmation that records of distribution are kept, and how often and under what circumstances an element is changed.	III.1 (1.2(b)), IV.4
C3	The security measures for storing illegal substances: substances to be tested, leftover residues not to be tested, substances awaiting disposal, and substances left behind by clients; and the access controls, including the persons with access to any stored substance.	III.1 (1.7(b)), III.3
C4	A floor plan showing the configuration of the site, including the room where the device or instrument is used, with all security measures labelled on it (access-controlled doors, the safe, other physical security features). A simple diagram is sufficient.	III.3, IV.3

D. Reference standards and registered test kits (if applicable)

Item	Health Canada asks for	Where in this guide
D1	Whether reference standards or registered test kits that are controlled substances would be used for drug checking with the proposed device or instrument. If not, skip to section E.	Not applicable to the Scatr method, which uses no reagents, reference standards or test kits. Test strips are treated as different from registered test kits and need their own policy if offered (IV.3).
D2	If so, for each controlled substance to be purchased: the name, the quantity, the name and address of the commercial supplier, and the catalogue number or registered test kit number.	Not applicable

E. Illegal substances

Item	Health Canada asks for	Where in this guide
E1	Policies and procedures describing the steps involved in the preparation and analysis of illegal substances, including the methods and instrumentation used.	III.1 (1.3, 1.5), IV.3
E2	A description of the data that would be collected on drug checking, and how results would be communicated to the individual who provided the sample.	III.1 (1.1, 1.5), IV.3, V.1
E3	If results are disclosed to the individual: confirmation that a disclaimer on the limitations of the drug checking method is discussed with them, and that harm reduction messaging, including the option to discard the substance, is also discussed.	III.1 (1.4), III.2
E4	Policies and procedures for the disposal of samples tested, and of any discarded samples that would not be analysed.	III.1 (1.7(d) to (f)), IV.5
E5	A description or copy of the record-keeping forms for documenting the disposal of substances tested, residues not tested, and substances left behind. Disposal records must be kept at the site and must include the fields in the table below, depending on the disposal method.	III.6, IV.5
E6	Policies and procedures to prevent the loss or theft of illegal substances at the site. The Office of Controlled Substances and the police must be informed within 24 hours of any loss or theft.	III.1 (1.8), V.2
E7	A description or copy of the record-keeping form for documenting and reporting any loss or theft. The Health Canada Loss or Theft Report Form may be used, or your own form.	III.1 (1.8), V.2, Appendix G
E8	Confirmation in the policies and procedures that records would be kept at the site for a minimum of 2 years, or for the duration of the exemption, whichever is longer.	III.1 (1.1(a)), V.6

Disposal record fields required by Health Canada, by method:

Transferred to police for destruction	Transferred to a licensed dealer (licensed for destruction)	Destroyed on site
The date the substance was stored. The sample identification code. The name of the staff member who packaged and stored the substance. The date the local police were contacted to request removal (within 24 hours, or at the frequency agreed with local law enforcement). The name and signature, or badge number, of the officer who picked up the substance. The date the substance was removed from the site by the officer.	The date the substance was stored. The sample identification code. The name of the staff member who packaged and stored the substance. The date the licensed dealer was contacted to request removal (within 24 hours). The name of the staff member who removed the substance from storage. The name of the licensed dealer who picked up the substance. The date the substance was removed from the site by the licensed dealer.	The sample identification code. The date of destruction. The name and signature of the staff member who destroyed the substance and of a witness, one of whom must be the RPIC or an A/RPIC. A statement that the illegal substance destroyed has been altered or denatured to such an extent that its consumption has been rendered impossible or improbable. If the substance was stored before destruction, the record must also give the date it was stored and the name of the staff member who packaged and stored it.

F. Substance transportation (if applicable)

Transport of illegal substances can be authorized for drug checking or disposal, but only between the site and a Health Canada laboratory, a licensed dealer, a section 56.1 exemption holder or a subsection 56(1) exemption holder. Western University is a subsection 56(1) exemption holder (24-103323-618), so samples sent to Western for confirmation testing fall under this section.

Item	Health Canada asks for	Where in this guide
F1	A description of the method of transport to or from the site, including who would transport (a commercial courier and/or an employee of either site) and how substances would be secured during transport. Canada Post letter mail is not an acceptable method.	III.1 (1.7(a), 1.7(c)), III.5
F2	A description or copy of the record-keeping form(s) documenting the chain of possession between individuals during transport, with the fields in the table below. If samples in a received package must be stored before analysis, the record must also give the name of the staff member who stored them and the name of the staff member who later removed them from storage.	III.5, IV.3

Chain of possession fields required by Health Canada:

Sending samples to another site	Accepting samples from another site
The substance identification code. The date the substance was stored. The name of the staff member who packaged and stored the substance. The date the substance was removed from the site for transport. The name of the staff member who removed the substance from storage and handed it to the transporter. The name and signature of the transporter who removed the substance from the site, or the name of the commercial courier service and the package tracking number.	The date the package was received. The substance identification code(s) in the received package. The name and signature of the transporter, or the name of the commercial courier service and the package tracking number. The name of the staff member who received the package.

G. Mobile (if applicable)

Item	Health Canada asks for	Where in this guide
G1	A description of the mobile unit (van, trailer, tent, camper, RV or motorhome, or other vehicle).	III.4, III.8.1
G2	The address where the mobile unit would be parked or stored outside operating hours.	II.5, II.7, IV.2
G3	The route plan: the neighbourhoods and/or specific addresses of stops, and/or the geographic range within which the mobile would operate.	III.8.2, III.8.3
G4	A description of how clients would be notified of the route, stop arrival and departure times, and any changes to the schedule.	III.8.4
G5	The contingency plan if the vehicle has mechanical issues and is out of service.	IV.2
G6	Measures for securely storing and disposing of illegal substances (items E3 and E4), with confirmation that illegal substances would not be stored in the mobile unit outside operating hours.	III.3, III.8, IV.2
G7	Confirmation of support from the property owner for each mobile stop: the municipal government for stops on public property, and the property owner for stops on private property.	III.8.2, IV.2
G8	Confirmation of whether the vehicle would stop within 200 m of any school or daycare; if yes, their names and the engagement activities the organization has conducted or would conduct with them.	IV.2
G9	Measures to address high client volumes.	IV.2

H. Application form

Complete Sections 1, 2 and 7.1 of the application form for an exemption to use a controlled substance for scientific purposes.

Item	Health Canada asks for	Where in this guide
H1	Section 1: check "New".	II.2
H2	Section 2: the name and contact information of the "Principal Investigator", who is the proposed applicant and exemption holder, and the name and email address of an alternate operational contact. Provide the name of the drug checking site or service in the "Institution/Company" box in Section 2.3, and the address of the site at which the service would be offered.	II.3, II.4, II.5
H3	Section 7: the applicant is the individual legally responsible for the site and must submit the completed declaration in Section 7.1.	II.14

Applications and questions go to exemption@hc-sc.gc.ca.

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